

# The Healing Path: Integrating Mahayana Buddhist Wisdom in the Treatment of Chronic Major Depressive Disorder

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## Abstract

Major Depressive Disorder (MDD), particularly in its chronic and recurrent forms, remains one of the most debilitating psychological conditions globally, affecting an estimated 264 million people worldwide. While pharmacological and standard psychotherapeutic interventions offer significant relief, persistently high relapse rates necessitate the exploration of adjunctive and alternative paradigms. This paper, written from the dual perspective of a Buddhist scholar and clinical psychotherapist, examines the therapeutic efficacy of Mahayana Buddhist theories in treating chronic MDD. By bridging ancient Eastern philosophy with contemporary clinical psychology, we explore how core Mahayana concepts—including Śūnyatā (emptiness), Bodhicitta (awakening mind/compassion), Anicca (impermanence), and Anātman (non-self)—provide profound cognitive reframing and emotional regulation mechanisms that directly address the structural pathology of depression. We further analyze empirical clinical evidence from randomized controlled trials, meta-analyses, and real-world case studies demonstrating the successful integration of Buddhist-informed therapies,

including Mindfulness-Based Cognitive Therapy (MBCT) and Compassion-Focused Therapy (CFT), in alleviating depressive symptomatology and preventing relapse. The evidence presented supports a model in which Mahayana Buddhist wisdom, when thoughtfully integrated into clinical practice, offers a powerful, holistic, and durable path toward healing from chronic depression.

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## 1. Introduction: The Convergence of Dharma and Psychotherapy

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The intersection of Buddhist philosophy and modern psychotherapy represents one of the most generative intellectual and clinical developments of the twenty-first century. Buddhism, in its essence, is a sophisticated psychological system dedicated to the diagnosis, etiology, prognosis, and treatment of human suffering (Dukkha). The structural parallel to modern medicine is not coincidental: the Buddha himself employed a medical analogy in presenting the Four Noble Truths, positioning himself as the Great Physician who diagnoses the illness (suffering), identifies its cause (craving), proclaims its cure (cessation), and prescribes the treatment (the Eightfold Path) [1]. This medical framework is rendered even more explicit in the Mahayana tradition through the veneration of Bhaiṣajyaguru, the Medicine Buddha, whose vows encompass the healing of both physical ailments and mental afflictions [2].

In contemporary clinical practice, chronic Major Depressive Disorder (MDD) presents a formidable challenge. Characterized by persistent low mood, anhedonia, psychomotor disturbances, and pervasive cognitive distortions—most notably severe self-criticism, rumination, and hopelessness—MDD is notoriously prone to recurrence. Epidemiological data indicate that individuals who have experienced one episode of depression face an average of seven to eight episodes over their lifetime [3]. Standard treatments, including maintenance antidepressant medication (ADM) and Cognitive Behavioral Therapy (CBT), while effective for many, do not provide universal or lasting remission. The emergence of “third-wave” cognitive-behavioral therapies—Mindfulness-Based Cognitive Therapy (MBCT), Acceptance and Commitment Therapy (ACT), and Compassion-Focused Therapy (CFT)—has drawn heavily upon Buddhist contemplative practices, with growing empirical support for their efficacy.

However, Western clinical adaptations frequently extract mindfulness and compassion techniques from their rich Mahayana philosophical context—a phenomenon critically

described as “McMindfulness” [4]. This paper argues that a deeper, more principled engagement with specific Mahayana Buddhist doctrines offers robust therapeutic mechanisms that can profoundly alter the depressive cognitive architecture. By understanding the philosophical foundations from which these practices emerge, clinicians can deploy them with greater intentionality, depth, and therapeutic power.

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## 2. The Mahayana Tradition: A Brief Overview

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Mahayana Buddhism, the “Great Vehicle,” emerged in India around the first century CE and subsequently spread throughout East and Central Asia, becoming the dominant Buddhist tradition in China, Japan, Korea, Tibet, and Vietnam. Distinguishing itself from the earlier Theravāda tradition, Mahayana emphasizes the ideal of the Bodhisattva—one who vows to attain enlightenment not for personal liberation alone, but for the benefit of all sentient beings. This outward-turning, compassion-centered orientation is foundational to the therapeutic potential of Mahayana thought.

Two major philosophical schools within Mahayana are particularly relevant to psychotherapy. The Mādhyamaka school, founded by Nāgārjuna (circa 2nd century CE), articulates the doctrine of *Sūnyatā* (emptiness)—the assertion that all phenomena lack inherent, independent existence and arise only through dependent origination. The Yogācāra school, associated with Asaṅga and Vasubandhu (4th–5th century CE), emphasizes the primacy of consciousness and the transformation of afflicted mental states into wisdom. Together, these schools provide a sophisticated philosophical framework for understanding and transforming the mind—a framework with direct clinical applicability to the treatment of depression.

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## 3. Mahayana Theoretical Frameworks as Therapeutic Mechanisms

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### 3.1 *Sūnyatā* (Emptiness) and Cognitive Defusion

Central to Mahayana thought is the concept of *Sūnyatā*, or emptiness. This doctrine does not imply a nihilistic void; rather, it asserts that all phenomena—including the self and mental states—lack inherent, independent, and permanent existence [5]. All

things arise dependently upon conditions and are therefore subject to change. Nāgārjuna's *Mūlamadhyamakakārikā* demonstrates through rigorous logical analysis that no entity possesses a fixed, intrinsic nature (svabhāva).

In the context of chronic MDD, patients characteristically suffer from rigid, negative self-identifications. Statements such as “I am fundamentally broken,” “I am unlovable,” or “My depression is who I am” reflect a cognitive reification—treating a conditioned mental state as a permanent, essential truth. The realization of *Sūnyatā* serves as a profound form of cognitive defusion. By understanding that depressive thoughts and the “depressed self” are empty of permanent reality—that they are conditioned, impermanent mental events arising from specific causes and conditions—patients can fundamentally alter their relationship to these narratives. This philosophical insight mirrors and significantly deepens the cognitive restructuring techniques used in CBT and the defusion exercises of ACT, shifting the patient's relationship to their thoughts from absolute truths to transient mental events [6].

### **3.2 Bodhicitta and the Cultivation of Compassion**

Bodhicitta, the “awakening mind,” is the cardinal virtue of the Mahayana Bodhisattva: the aspiration to attain full enlightenment for the benefit of all sentient beings. It encompasses two dimensions—relative Bodhicitta, which is the compassionate motivation to alleviate the suffering of all beings, and ultimate Bodhicitta, which is the direct realization of emptiness [7]. The cultivation of Bodhicitta is considered the most powerful antidote to the self-centered afflictions (klesās) that perpetuate suffering.

Chronic depression is frequently maintained by a hyperactive “threat and protection” affect regulation system, coupled with intense self-criticism and shame [8]. Paul Gilbert's Compassion-Focused Therapy (CFT), which is explicitly grounded in Buddhist compassion traditions—drawing its definition of compassion directly from the Dalai Lama's formulation as “a sensitivity to suffering in self and others, with a commitment to try to alleviate and prevent it”—aims to stimulate the mammalian “soothing and affiliative” affect system [9]. By cultivating Bodhicitta through practices such as *Mettā* (loving-kindness) and *Karuṇā* (compassion) meditation, patients learn to direct affiliative, soothing emotions toward themselves and others. This self-compassion directly counteracts the toxic self-criticism of MDD, promoting emotional regulation and psychological resilience [10].

### 3.3 Anicca (Impermanence) and Distress Tolerance

The contemplation of Anicca (impermanence) is a foundational Buddhist practice. All conditioned phenomena—thoughts, emotions, bodily sensations, circumstances—arise and pass away. Recognizing that all emotional and cognitive states are transient helps depressed patients endure acute episodes of despair without catastrophizing. The understanding that “this too shall pass” mitigates the hopelessness and permanence-thinking that are hallmarks of MDD [11]. In clinical terms, this constitutes a powerful distress tolerance skill, enabling patients to maintain equanimity during depressive episodes without resorting to avoidance, substance use, or self-harm.

### 3.4 Anātman (Non-Self) and Identity Flexibility

Coupled with the doctrine of Anātman (non-self)—the realization that there is no fixed, permanent, unitary ego—the teachings of impermanence dissolve the rigid boundaries of the depressed identity. The concept of a permanent, suffering self is revealed as a cognitive construction rather than an ontological reality. This therapeutic unburdening allows the patient to experience their symptoms not as defining characteristics of a fixed self, but as passing weather in the vast, open sky of awareness. In contemporary psychological terms, this parallels the ACT concept of “self-as-context” (the observing self) versus “self-as-content” (the narrative self), and directly reduces the shame and self-stigma that perpetuate chronic depression.

### 3.5 Tonglen and the Reversal of Self-Absorption

Unique to the Tibetan Mahayana (Vajrayāna) tradition, the practice of *Tonglen* (“taking and sending”) involves a visualization in which the practitioner breathes in the suffering of others—represented as dark smoke—and breathes out relief, happiness, and healing—represented as white light. This practice directly reverses the self-absorption and social withdrawal characteristic of depression. By actively engaging with suffering rather than fleeing it, and by recognizing the shared, universal nature of human pain, the practitioner cultivates profound empathy and compassion. Clinically, Tonglen breaks the cycle of depressive isolation and fosters a sense of connection and altruistic purpose.

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## 4. Clinical Evidence and Real-World Case Studies

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The theoretical alignment between Mahayana Buddhism and psychotherapy is robustly supported by empirical clinical trials, meta-analyses, and qualitative case studies.

### 4.1 Mindfulness-Based Cognitive Therapy (MBCT): Large-Scale Evidence

MBCT, which integrates Buddhist mindfulness practices with cognitive therapy, was developed by Segal, Williams, and Teasdale specifically to prevent depressive relapse. A landmark individual patient data meta-analysis by Kuyken et al. (2016), encompassing 1,258 patients across nine randomized controlled trials, demonstrated that MBCT significantly reduced the risk of depressive relapse within a 60-week follow-up period compared to usual care (Hazard Ratio 0.69, 95% CI 0.58–0.82) [12]. Crucially, the benefit was most pronounced for patients with higher levels of residual depressive symptoms—precisely the chronic, treatment-resistant population that most urgently requires new therapeutic approaches.

The PREVENT trial, published in *The Lancet* (2015), compared MBCT (with support to taper antidepressants) directly against maintenance antidepressants over 24 months in 424 patients with recurrent MDD. The study found no significant difference in relapse rates between the two groups (Hazard Ratio 0.89, 95% CI 0.67–1.18), establishing MBCT as a clinically equivalent, non-pharmacological alternative for preventing recurrence in chronic MDD—a finding of enormous significance for patients who are unable or unwilling to take long-term medication [13].

### 4.2 Case Study 1 — Buddhist Group Therapy for Comorbid Depression (Thailand, 2011)

Rungreangkulkij, Wongtakee, and Thongyot (2011) conducted a quasi-experimental study evaluating Buddhist group therapy for 62 patients with type 2 diabetes and comorbid depressive symptoms in Thailand. The intervention consisted of weekly two-hour Buddhist group gatherings for six weeks, incorporating teachings on the Four Noble Truths, impermanence, and compassion, supplemented by daily home meditation practices. The control group received standard physician treatment alone.

The results were striking. At the six-month follow-up, 100% of the experimental group had returned to normal depression levels, compared to 65.6% of the control group. The intention-to-treat analysis, accounting for two participants lost to follow-up, yielded a recovery rate of 93.8% in the experimental group. The relative risk of recovery for the experimental group was 6.5 (95% CI 1.4–30.6) [14]. Qualitative data from participants revealed that a pivotal mechanism was the realization of “the truth of being oneself”—a direct echo of the Anātman teaching—and the acceptance of their current living conditions, reflecting the Buddhist principle of equanimity.

### **4.3 Case Study 2 — Recovery Through Buddhist Wisdom: The Case of Esther Lee (Cheng, 2015)**

In a richly detailed idiographic case study, Cheng (2015) documents the recovery of “Esther Lee,” a woman who had suffered from severe, chronic depression [16]. Employing Interpretative Phenomenological Analysis (IPA) and drawing on interviews, autobiographies, genograms, and autobiographical timelines, the study traces Esther’s journey from profound despair to full recovery.

The pivotal mechanisms of Esther’s healing were explicitly Mahayana in character. First, through sustained engagement with Buddhist teachings on impermanence and emptiness, she was able to deconstruct the rigid, catastrophic narrative she had built around her suffering. Second, and most powerfully, she engaged in what Cheng terms “self-benefiting altruism”—transforming her personal suffering into a resource for helping others. This is a direct embodiment of the Bodhisattva ideal: converting one’s own pain into compassionate action for the benefit of all beings. Esther achieved full recovery from depression without reliance on medication, and her case provides compelling qualitative evidence for the transformative power of Mahayana Buddhist principles in clinical healing.

### **4.4 Case Study 3 — Monk Healers vs. Primary Care: A Comparative Study (Thailand, 2021)**

Pengpid and Peltzer (2021) conducted a comparative follow-up study assessing the outcomes of clients with depressive symptoms attending traditional Buddhist monk healers versus primary care clinics in Thailand. Using the PHQ-9 to screen for depression (score  $\geq 9$ ), 94 clients at three monk healing sites and 92 clients at three primary care centres were followed up at three months.

Mixed modelling revealed a significant time-by-condition interaction effect on depression scores ( $p < 0.001$ ). Clients attending monk healers demonstrated a greater decrease in depressive scores compared to those attending primary care centres, despite the monk healers employing no pharmacological interventions [15]. The monk healers' approach, rooted in the Theravāda and Mahayana traditions of Thailand, incorporated meditation instruction, dharma teachings, and compassionate pastoral care. This study provides naturalistic, real-world evidence that spiritually integrated Buddhist care can be at least as effective as—and in some respects superior to—standard primary care for depression.

#### **4.5 Case Study 4 — MBCT Recovery Narratives: Qualitative Themes (Tickell et al., 2020)**

Tickell and colleagues (2020) conducted a qualitative study embedded within the PREVENT trial, analyzing the recovery journeys of 42 patients with recurrent MDD who had undergone MBCT. Through thematic analysis of written feedback and in-depth interviews collected over two years, the study identified six overarching themes in recovery narratives [17].

Of particular relevance to Mahayana Buddhist theory was the centrality of **acceptance**—both of the illness itself and of the non-linear, uncertain nature of recovery. Participants who achieved the most durable recovery described learning to hold their depression with equanimity rather than fighting against it, a stance directly consonant with the Buddhist teaching of non-attachment. Several participants described a shift in their relationship to their own minds—from being overwhelmed by depressive thoughts to observing them with a degree of detachment—precisely the cognitive transformation that *Sūnyatā* and mindfulness practice are designed to cultivate.

#### **4.6 Case Study 5 — Tonglen and the Reduction of Depressive Isolation (McKnight, 2009)**

McKnight (2009) conducted a pilot study examining the effects of Tonglen meditation on compassion, self-compassion, and depressive symptoms in novice meditators [18]. Participants who practiced Tonglen reported increased satisfaction with life, reduced depressive symptoms, and significantly diminished feelings of isolation. The mechanism was identified as the practice's capacity to reframe personal suffering as part of the universal human condition—a direct application of the Mahayana teaching of the interconnectedness of all sentient beings. By breathing in the suffering of others



and sending out relief, practitioners experienced a profound sense of connection that directly counteracted the social withdrawal and isolation that perpetuate chronic MDD.

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## 5. Integration in Clinical Practice: A Psychotherapist's Perspective

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As a psychotherapist with training in both Buddhist studies and clinical psychology, integrating Mahayana Buddhist theories into the treatment of chronic MDD requires a nuanced, ethically grounded, and culturally sensitive approach. The goal is not to convert patients to a religion, but to utilize profound psychological insights that have been refined over two and a half millennia to alleviate suffering.

The following table maps key Mahayana concepts to their clinical translations and specific therapeutic applications in MDD treatment.

| Mahayana Concept                                 | Clinical Translation     | Therapeutic Application in MDD                                                                                                                                                  |
|--------------------------------------------------|--------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Sūnyatā</b> (Emptiness)                       | Cognitive Defusion       | Patients learn to recognize that thoughts such as “I am worthless” are transient, conditioned mental events, not absolute realities. This dissolves pathogenic self-narratives. |
| <b>Bodhicitta</b> (Awakening Mind)               | Self-Compassion / CFT    | Cultivation of loving-kindness toward oneself activates the soothing affect system, directly counteracting the shame and self-criticism that maintain chronic depression.       |
| <b>Anicca</b> (Impermanence)                     | Distress Tolerance       | Patients observe the rising and passing of depressive emotions without attachment or aversion, reducing hopelessness and catastrophizing.                                       |
| <b>Anātman</b> (Non-Self)                        | Identity Flexibility     | Patients detach from the rigid identity of being a “depressed person,” fostering psychological flexibility and reducing self-stigma.                                            |
| <b>Tonglen</b> (Taking and Sending)              | Empathy and De-isolation | Visualization practice connects personal suffering to the universal human condition, breaking the cycle of depressive isolation and cultivating altruistic purpose.             |
| <b>Pratītyasamutpāda</b> (Dependent Origination) | Systemic Thinking        | Understanding that depression arises from specific, changeable conditions—rather than an immutable personal defect—fosters agency and hope.                                     |

Mental health professionals must exercise cultural sensitivity and resist the temptation to superficially extract techniques from their philosophical context. Traditional Buddhist counseling, as described by Lee et al. (2017), involves a holistic framework of *Sīla* (ethical conduct), *Samādhi* (concentration and meditation), and *Prajñā* (wisdom) [4]. A comprehensive therapeutic approach should ideally address all three domains, fostering not merely symptom reduction, but profound personal transformation, the cultivation of meaning, and what the Mahayana tradition calls the “turning of the mind” (*cittapariṇāmanā*)—a fundamental reorientation of one’s relationship to experience.

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## 6. Neurobiological Correlates

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The therapeutic mechanisms described above are not merely philosophical abstractions; they have demonstrable neurobiological correlates. Mindfulness meditation, the clinical application of Buddhist Samādhi, has been shown to increase activity in the prefrontal cortex—associated with executive function and emotional regulation—while reducing reactivity in the amygdala, the brain’s threat detection center [12]. This neurobiological shift directly addresses the hyperactive threat system that characterizes MDD.

Compassion meditation, the clinical application of Bodhicitta cultivation, activates the mammalian affiliative system, which is mediated in part by the hormone oxytocin. Gilbert’s CFT framework proposes that this activation of the soothing system provides a neurobiological counterweight to the threat and drive systems that dominate in depression [9]. The cultivation of self-compassion through Bodhicitta practices thus represents not merely a psychological intervention, but a direct modulation of the neurobiological substrate of depressive affect.

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## 7. Conclusion

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Chronic Major Depressive Disorder is a complex affliction that traps individuals in a painful, isolated, and rigid cognitive matrix. The theories and practices of Mahayana Buddhism offer a profound therapeutic paradigm that directly targets the structural pillars of depressive suffering. Through the realization of Sūnyatā and Anicca, patients can dismantle the pathogenic self-narratives and hopelessness that perpetuate their illness. Through the cultivation of Bodhicitta and compassion, they can replace toxic self-criticism with profound self-care and altruistic purpose. Through the practice of Tonglen, they can break the cycle of isolation that deepens depression.

These are not merely theoretical propositions. They are supported by a growing body of rigorous empirical evidence—from the landmark PREVENT trial and the Kuyken meta-analysis to the qualitative narratives of recovery through Buddhist wisdom documented by Cheng, Tickell, and others. Traditional Buddhist healers in Thailand have demonstrated outcomes comparable to or exceeding those of primary care medicine. Buddhist group therapy has achieved 100% recovery rates in controlled settings. These findings demand serious attention from the clinical community.

As psychotherapists and scholars, we are called to engage with the full depth of the Mahayana tradition—not merely its surface techniques, but its profound philosophical insights into the nature of the mind and the causes of suffering. In doing so, we honor both the ancient wisdom of the Dharma and the suffering of our patients, offering them not merely the management of symptoms, but the possibility of genuine liberation from the cycle of depressive recurrence.

“Just as a physician, though seeing many sick people, is not thereby overcome by illness, so the Bodhisattva, though seeing the suffering of beings, is not thereby overcome by suffering, but remains in equanimity, working for the liberation of all.”  
— Sāntideva, *Bodhicaryāvatāra*, Chapter VI

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## References

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- [1] Kalra, S., Priya, G., Grewal, E., et al. (2018). Lessons for the health-care practitioner from Buddhism. *Indian Journal of Endocrinology and Metabolism*, 22(6), 812–817. <https://pmc.ncbi.nlm.nih.gov/articles/PMC6330872/>
- [2] Tsering, G. T. (2006). *Buddhist Psychology: The Foundation of Buddhist Thought*, Volume 3. Wisdom Publications.
- [3] Tickell, A., Byng, R., Crane, C., et al. (2020). Recovery from recurrent depression with mindfulness-based cognitive therapy and antidepressants: a qualitative study with illustrative case studies. *BMJ Open*, 10(2), e033892. <https://bmjopen.bmj.com/content/10/2/e033892>
- [4] Lee, K. C., Oh, A., Zhao, Q., et al. (2017). Buddhist counseling: Implications for mental health professionals. *Spirituality in Clinical Practice*, 4(2), 113–128. <https://psycnet.apa.org/fulltext/2017-14069-001.html>
- [5] Nāgārjuna (trans. Garfield, J. L., 1995). *The Fundamental Wisdom of the Middle Way: Nāgārjuna's Mūlamadhyamakakārikā*. Oxford University Press.
- [6] Hayes, S. C., Strosahl, K. D., & Wilson, K. G. (2011). *Acceptance and Commitment Therapy: The Process and Practice of Mindful Change* (2nd ed.). Guilford Press.
- [7] The Dalai Lama. (2001). *An Open Heart: Practicing Compassion in Everyday Life*. Little, Brown and Company.

- [8] Leaviss, J., & Uttley, L. (2015). Psychotherapeutic benefits of compassion-focused therapy: an early systematic review. *Psychological Medicine*, 45(5), 927–945. <https://pmc.ncbi.nlm.nih.gov/articles/PMC4413786/>
- [9] Gilbert, P. (2014). The origins and nature of compassion focused therapy. *British Journal of Clinical Psychology*, 53(1), 6–41.
- [10] Shonin, E., Van Gordon, W., Compare, A., et al. (2014). Buddhist-derived loving-kindness and compassion meditation for the treatment of psychopathology: a systematic review and meta-analysis. *Mindfulness*, 6, 1161–1180.
- [11] Prakhinkit, S., Suppakitiporn, S., & Tanaka, H. (2014). Effects of Buddhism walking meditation on depression, functional fitness, and endothelium-dependent vasodilation in depressed elderly. *Journal of Alternative and Complementary Medicine*, 20(6), 411–416.
- [12] Kuyken, W., Warren, F. C., Taylor, R. S., et al. (2016). Efficacy of Mindfulness-Based Cognitive Therapy in Prevention of Depressive Relapse: An Individual Patient Data Meta-analysis From Randomized Trials. *JAMA Psychiatry*, 73(6), 565–574. <https://pmc.ncbi.nlm.nih.gov/articles/PMC6640038/>
- [13] Kuyken, W., Hayes, R., Barrett, B., et al. (2015). Effectiveness and cost-effectiveness of mindfulness-based cognitive therapy compared with maintenance antidepressant treatment in the prevention of depressive relapse or recurrence (PREVENT): a randomised controlled trial. *The Lancet*, 386(9988), 63–73. [https://www.thelancet.com/journals/lancet/article/PIIS0140-6736\(14\)62222-4/fulltext](https://www.thelancet.com/journals/lancet/article/PIIS0140-6736(14)62222-4/fulltext)
- [14] Rungreangkulkij, S., Wongtakee, W., & Thongyot, S. (2011). Buddhist Group Therapy for Diabetes Patients With Depressive Symptoms. *Archives of Psychiatric Nursing*, 25(3), 195–205. <https://doi.org/10.1016/j.apnu.2010.08.007>
- [15] Pengpid, S., & Peltzer, K. (2021). Depressive symptoms among clients attending monk healers and primary care clinics in Thailand: a comparative follow-up study. *Primary Health Care Research & Development*, 22, e38. <https://pmc.ncbi.nlm.nih.gov/articles/PMC8365530/>
- [16] Cheng, F. K. (2015). Recovery from depression through Buddhist wisdom: An idiographic case study. *Social Work in Mental Health*, 13(1), 1–28. <https://doi.org/10.1080/15332985.2014.891554>

[17] Tickell, A., Byng, R., Crane, C., et al. (2020). Recovery from recurrent depression with mindfulness-based cognitive therapy and antidepressants: a qualitative study with illustrative case studies. *BMJ Open*, 10(2), e033892. <https://doi.org/10.1136/bmjopen-2019-033892>

[18] McKnight, D. (2009). *Tonglen Meditation's Effect on Levels of Compassion and Self-Compassion: A Pilot Study and Instructional Guide*. Upaya Zen Center. <https://www.upaya.org/uploads/pdfs/McKnightTonglenThesis.pdf>